# 15.4 Ectopic Pregnancy

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "I have sharp pain on one side and spotting."<br>• The pain goes up to her shoulder<br>**Objective**<br>• Pregnancy test positive, but the scan shows an empty uterus<br>• Tender over one adnexa<br>• Pain, 8/10 | **ACTUAL**<br>**Acute pain** related to tubal distension or rupture as evidenced by unilateral pelvic pain and shoulder-tip pain. | Pain controlled before and after treatment. | • Give analgesia as prescribed.<br>• Nothing by mouth; get her ready for surgery or methotrexate as ordered.<br>• Keep checking what the pain is like and report any sudden worsening. | Pain controlled: **goal met**. |
| **Subjective**<br>• "Will I be able to get pregnant again?"<br>**Objective**<br>• Tearful<br>• Frightened | **ACTUAL**<br>**Fear** related to loss of pregnancy and future fertility as evidenced by expressed fear and tearfulness. | Talks about her feelings and understands what this means for future pregnancies. | • Give her honest information together with the doctor.<br>• Give her space to grieve.<br>• Book follow-up. | Shows she understands: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Pregnancy in the tube | **RISK**<br>**Risk for shock** as evidenced by risk factors: possible tubal rupture with intra-abdominal bleeding. | Rupture picked up early. | • Vital signs every 15–30 minutes.<br>• Put in large-bore IV access.<br>• Report low BP, a fast heart rate or fainting at once. | Stable: **goal met**. |